



Patient Property in Mental Health Wards Procedure

1. Scope

Site	Operational Area	Applicable to
Armadale Health Service	Mental Health	All disciplines

2. Purpose

This document outlines the procedure for managing patient property in the inpatient mental health wards. Property is defined as any article in a person's possession upon their admission and whilst an inpatient of the Armadale Mental Health Service (AMHS).

3. General considerations

AMHS staff will appropriately record, store, and return patient's valuables and belongings. There will be a documented determination on the appropriateness of items to remain with the individual dependent on their mental state and the clinical environment

On admission all patients are to be advised that:

- Valuables should not be brought into the inpatient unit where possible. Where there is no alternative, they must be inventoried and stored in the safe.
- There should be a determination in conjunction with the patient as to whether jewellery, e.g., wedding rings, can be kept on their person.
- Prohibited items such as alcohol, illicit drugs, and weapons will be confiscated. The shift coordinator and treating doctor will determine if further action is required, e.g., security involvement. Please refer to the [EMHS Alcohol and Prohibited Substances Policy](#).
- AMHS does not take any responsibility for losses of property if not secured in accordance with these guidelines.
- Items identified as inappropriate to the inpatient environment will be documented in the Property Record Book on a numbered Patient Property Sheet (HA 00180) and stored until they are returned upon discharge/transfer and if appropriate on utilisation of leave. Location of these stored items is to be identified on the patient property sheet. Items that can remain with the patient may vary and will be dependent upon assessment of the patient and clinical judgement. This should be clearly documented in the clinical notes by the admitting nurse.

3.1 Responsibility

The AMHS will only assume responsibility in relation to patient property for all documented valuables which are held in the ward safe.

- Staff will recommend that all patients use secure storage facility for items considered valuable and for money over the value of \$20. The risk of retaining money on their person should be discussed and documented in the patient's health record.

- All patients will be encouraged to send valuables and excess money home with friends or relatives. Pending pick up, these items should be placed in the safe. The person receiving valuables to return home, along with consumer, should sign the bottom section of the property sheet when collecting these valuables.
- The admitting nurse for patients on Moodjar, Karri and Banksia Wards will advise patients that AMHS does not accept responsibility for any property which is retained by the patient on the ward.
- Duty of care for all patients' property ceases on discharge/transfer outside of the service or disposal of property in accordance with the WA Mental Health Act 2014 Section 259.
- For involuntary patients admitted to the AMHS with clear risk concerns regarding property and all patients admitted to Yorgum ward, a full inventory of items will be completed, and items deemed a risk to the patient or others will be stored in a secure cupboard.

Visual checks

The nursing admission process for all wards is to include a visual check of all patients' property. Property brought onto the ward from patients returning from leave, visitors or purchased and delivered from external providers are to also be subject to a visual check.

4. Restricted Items

The below categories of items are a guideline for clinicians assessing the suitability of personal items within the Armadale Mental Health Service.

4.1 Items deemed not appropriate for any wards

Lighters/matches/vape products/e-cigarettes	Razors/cutting or slicing devices	Prescribed and non-prescribed Medications
Glassware	Sharp items including Knives	Mirrors
Aerosol Sprays	Coat Hangers	

4.2 Additional Items deemed not appropriate for Yorgum Ward

Yorgum Ward [High Dependency Unit (HDU)] is a secure environment where patients who are particularly vulnerable and deemed high risk are admitted for reasons of safety. While it is an objective of the AMHS to use least restrictive options, items brought into this environment are subject to additional screening to assist staff in managing the safety of all patients in this environment.

Belts/cords in clothes/braces	Photographic equipment	Money in excess of AUD\$20.00
Hair ties/ties	Gym equipment	Laces/ribbons
High heel shoes	Cans	Pins/needles
Headphones with cords/chargers/cables/other cords (e.g. for mobiles/PC) with dimensions longer than 20cm and able to be tied in a knot	Boots with steel caps	
Ask patient to remove outer layer of clothing, turn out pockets, shoes and socks (if required)		

4.3 Items requiring a clinical decision

Computer equipment	Money (limited to AUD\$20.00)	Mobile phone (supervised)
Earrings	Nail enamel	Tampons
Hairclips	Necklaces	Watches
iPads / Android tablets	Rings	Makeup
Insulin Pumps	Razors (supervised)	Writing implements (pens/pencils)
Other items as deemed appropriate by staff		

5. High Risk Patients

Some patients may present as a high risk of ongoing suicide, self-harm or illicit substance use risk. To support these individuals on their road to recovery additional actions are required to ensure their safety during this period. This risk is to be identified based on the clients individual Risk Assessment and Management Plan (RAMP) with consideration given to PSOLIS Alerts regarding previous instances of attempted suicide, self-harm or illicit substance use whilst an inpatient.

5.1 Additional actions for patients identified as High Risk

- The patient is to be identified as a POC for their associated high risk on i.Clinical Manager. This is to then be updated on the ward report.
- The treating team are to discuss with the patient regarding their identified risk, ensuring a Consumer Safety Plan is completed and having a frank conversation about actions being taken to ensure their safety.
- The RAMP is to be reviewed daily and updated as necessary
 - if reviewed and not requiring updating an entry is to be made on i.Clinical Manager and the DMR stating “RAMP reviewed and current.”
- [Safe and Supportive Observations](#) are to be considered in the management of the patient’s risk.
- All visual checks of property being brought to the ward, as above, is to be documented in the DMR.
- At a minimum the patient’s environment will be examined every 7 days for any restricted or other hazardous items to ensure that they continue to be nursed in a safe environment.

6. Valuables

- Valuables include jewellery, wallets, bank notes, mobile phones, coins, keys, traveller’s or other cheques, passports, bank account cards, life insurance policies, wills and other items which are of significant financial or sentimental value to the patient or other person.
- If an item of clothing is valued by the patient, it must be recorded and stored.
- Valuables are not deemed to include hearing aids, spectacles, and dentures. These items are ONLY recorded in the patient property sheet if they are removed from the patient’s possession and locked away in the patient property cupboard, due to patient safety, under the sections allocated to ‘Activities of Daily Living: sight, hearing, and teeth/dentures.

7. Storage of Property

- Valuables are to be recorded in the Property Record Book, a copy is given to the patient and a copy is filed in the patient health care record. Clothing is not to be recorded and stored unless specifically requested by the patient. Patients admitted to Yorgum or as an involuntary patient on Banksia Ward require clothing to be recorded and stored.
- Valuables are stored in a plastic box with the patient’s identification sticker. This box is stored in either a locked cupboard on the relevant ward, or in the safe located in the Leschen reception area. Location of this box will be documented on the appropriate Property Record Sheet.

8. Mobile Phones

- Use of any photographic equipment by patients is prohibited. Patients who possess a mobile phone with the capacity to take and store photographs are to be advised that they are not to use this facility whilst on the ward or within the mental health unit, building.
- Use may be proactively prevented by confiscation of the equipment for the time the person is within the mental health, for guidance please see [Section 10](#).
- Mobile phones can be retained by patients on all wards at the discretion of clinical staff The status quo should be that patients have access to all means of communication unless it is

determined that the rules of confidentiality within a ward environment are being breached or there is misuse of certain functions of the phone.

Before a mobile phone is confiscated the following needs to be considered:

- Patients have a right to private, lawful, and uncensored communication, such as being able to see and speak to others, writing and receiving letters, making and receiving phone calls, communicating through electronic means such as email or Facebook, and being visited or contacted by others including a legal representative.
- The major role played by social media in the lives of patients should be recognised and for therapeutic reasons patients should have the same access to social media as any other person in society with the ability to record and take photos, this should not automatically diminish patient access to their phones. Restrictions should only be used if there is evidence that there is improper use.

A psychiatrist can prohibit or limit this right to access means of communication if they are satisfied that it would not be in the patient's best interests for them to exercise this right.

Restrictions of communication can apply to voluntary patients, persons referred for examination, involuntary patients, and mentally impaired accused. To make an order to restrict access to a specific device or devices, the psychiatrist completes a Mental Health Act 2014 (MHA 2014) Form 12C indicating the reason and gives a copy to the patient and any carer, close family member or support person of the patient.

The Mental Health Advocacy Service should be informed of the restriction within 24 hours. The restriction lasts up to 24 hours however the psychiatrist can review the restriction and decide to confirm, amend, or revoke the order before the time lapses (record of confirmation or revocation of restriction of freedom of communication – attachment to MHA 2014 Form 12C).

It is the responsibility of the psychiatrist to conduct this review; however, it is not mandatory to examine the patient. The psychiatrist can make his or her decision on information received from the clinical staff about whether the restriction should continue.

Any confirmation lasts for up to 24 hours and must then be reviewed with the psychiatrist having the same options as before.

9. Food

Consideration regarding patients bringing food into the inpatient units should be made on an individual basis. A determination should allow for the comfort, cultural beliefs, and customs where practicable and the dietary needs of the patients. This determination should be documented in the health record to ensure consistency between clinicians.

Clinical judgement and sensitivity should be exercised and also allowing for the proper storage, safe handling and consumption of food in line with safe food handling practices.

10. Searching a patient and their belongings

If there is reasonable concern that the patient may be in possession of items considered a danger to themselves or others, a search of that patient may be undertaken as per [EMHS Searching of Mental Health Patient Belongings and Property Policy](#).

Inpatients who are minors (under the age of 18 years) should not be searched except in the presence of a chaperone as per Section 6: Search of Minors of the [EMHS Searching of Mental Health Patient Belongings and Property Policy](#).

A person, who searches a person in accordance with the MHA 2014 must make a record of the search using MHA 2014 Form 8A – Record of Search and Seizure; and if any article(s) are seized, a record of the articles seized (s165).

When an article has been seized from a person during a search and seizure and is being stored/kept by the mental health service, the person in charge of the mental health service must ensure that a record of how a seized article was dealt with is made using MHA 2014 Form 8B – Record of Dealing with Seized Article. MHA 2014 Form 8B must be completed at the time when the article is 'dealt with' in one of the following ways:

- Returned to the person; or
- Returned to the person's personal support person; or
- Destroyed or otherwise disposed of (after having been stored for 6 months after the person left the mental health service).

10.1 Search involving the use of WA Police

- Police drug detection dogs will routinely patrol the wards. There will be signage in the areas to notify patients and visitors that this will be occurring. In agreement with the Mental Health Advocacy Service, the patient must be present to enter their room and for the canine to sniff the belongings.
- Should any substance be located, this will be dealt with by summons only and the police will have a portable video camera for evidence recording, police do not need a witness to undertake this process of interview. The police will not remove the patient from the organisation for any interview to occur.
- Should a visitor be present, and substances be located on their person or property, this is a police matter and AMHS have no requirement for any further details of the person or substance unless the service specifically believe that our patients are at immediate risk from the substance.
- Should there be an alert around substances on a staff member, this is both a police matter and organisational matter and should be dealt with in conjunction with the Service Director MH and Nurse Coordinator.
- Documentation in the health record that the drug detection dogs have been in the organisation should only occur in the individual patients' health record if there is an outcome regarding the location of substances, or distress or any other concern regarding the sniffer dogs specifically related to the patient.
- The Service Director Mental Health will be the primary contact with the police re routines of sniffer dogs in the service and any other required discussions with the police re processes, so please ensure any concerns are raised.
- The Service Director Mental Health will notify the Nurse Coordinator Mental Health to ensure the senior staff are aware of when the dogs will be on the campus.

11. Discharge and/or transfer

Upon transfer to a different ward within AMHS, if a Property Record Sheet has been completed then it must be signed by the staff and patient to indicate all property inclusive of medication has accompanied them on transfer.

If a patient is being transferred to Yorgum ward a new Property Record Sheet is to be completed by the admitting nurse to update currently held valuables.

Upon discharge/transfer from AMHS, all items of property listed on the Property Record Sheet must be returned to the patient. Exceptions to this might include:

- Medication that has passed its expiry date
- Medication the patient is no longer taking
- Any medication perceived to have a harmful effect on the patient
- Illegal weapons/objects
- Illicit substances.
- Any other property that is deemed to cause harm or concern to the patients' health and wellbeing.

These exemptions of return must be fully documented in the patient's healthcare record.

Other items of property held in safekeeping may be returned to the patient and/or carer in accordance with an assessment of the relevant risks involved. The discharging nurse will check the Property Record Book for items belonging to the patient in the safe and is to ensure that the store cupboard with patient property has been checked no property is left behind.

Staff can collect valuables from the safe between 9:00am-10:00am and 2:00pm-3:00pm Monday-Friday. After hours, the patient will accompany the discharge nurse and security staff to Leschen Reception to access valuables from the safe.

The discharging nurse is to ensure that the envelope is given to the patient intact and request that the patient checks the contents prior to signing the Property Record Book:

- Valuables must only be returned to the patient, or with permission, to their carer.
- Other persons may take receipt of the patient's valuables only if staff has received permission from the patient.
- If the patient is unable to make a decision, capacity should be stated and the next of kin/legal guardian may designate a representative to collect the stored items.
- The Mental Health Discharge Checklist PAKMR 920 is to be completed in relation to property being returned/destroyed.

11.1 Property left behind after discharge:

Initially the patient must be contacted to collect their belongings and an entry made in the health record that this has occurred.

The discharge nurse is to place belongings left behind by the patient in a suitable patient bag with a patient label attached in addition to the following:

- An inventory of the belongings must be recorded on a Property Record Sheet.
- The yellow copy will be placed inside the bag with the belongings.
- The white copy should be replaced in the health record with a remark that 'these belongings have been left behind'.
- The green copy will remain in the Property Record Book.

A letter will then be sent to the patient informing that they must make arrangements for the collection of their property within six (6) months, or they will be notified 30 days after that period, that their property will be destroyed in accordance with the MHA 2014 Section 259. Should items not be collected within 30 days, the remaining items should be provided to Service

Director Mental Health for determination of disposal. Should a passport be within the property, this should be returned to the Australian Passport Office.

12. Deceased patient property

Patient property is to be returned to the next of kin following the same process as the discharge/transfer process above.

The deceased patient's clothing that has been separated from the body is to be sealed in a secure double bag, clearly labelled with the deceased patient's details, and sent with the body to the Hospital Mortuary for transfer to the State Mortuary.

13. Loss of property

Loss of property will be investigated as per the [EMHS Theft and Loss Reporting Policy](#) with written feedback provide to the patient.

14. Legislative context

The following documents are required to give effect to this policy:

- *Mental Health Act 2014 (WA)*
- *Mental Health Regulations 2016 (WA)*
- *Disposal of Uncollected Goods Act 1970 (WA)*
- *Firearms Act 1973 (WA)*
- *Weapons Act 1999 (WA)*
- HDWA [Medicines Handling Policy MP0139/20](#)
- EMHS [Searching of Patients Belongings and Property Policy EMHS:153](#)
- EMHS [Security: Theft and Loss Reporting Policy EMHS:128](#)
- EMHS [Alcohol and Prohibited Substances Policy EMHS:112](#)
- EMHS [Rights of Patients in Mental Health Policy EMHS:170](#)

15. Supporting information

The following documents support the implementation of this policy:

- [Medication Management Procedure AKG-PRO-0255-18](#)
- [Death of a Patient AKG-POL-0388-14](#)
- [Alcohol, Illicit Substances, Non-Prescribed Substances Management on the Inpatient Wards Procedure MEN-PRO-0023-10](#)
- [AKG Safe and Supportive Observations: Procedure MEN-PRO-0310-16](#)
- [AKK Consumer Safety Plan MEN-PRO-0346-13](#)
- [AHS Clinical Handover Procedure \(AKG-PRO-0521-19\)](#)

16. Compliance, monitoring and evaluation

Compliance against this policy will be monitored by the Mental Health Safety and Quality Committee. Compliance monitoring activities may include:

- Healthcare record documentation audit relating to the management of patient property

- Review of patient feedback data relating to the management of patient property.

17. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.13 Feedback and complaints management

18. Policy owner

Enquiries relating to this policy may be directed to:

Title: Service Director Mental Health
 Department: Mental Health
 Email: Karen.Elliott@health.wa.gov.au

19. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4	12/04/2024	12/04/2027	Minor amendment.

20. Authorisation

This policy has been authorised and issued by the Service Director Mental Health.

Authorised by	Karen Elliott – Service Director Mental Health
Authorisation date	10/04/2024
Published date	12/04/2024